

Quest Diagnostics

LABORATORY REPORT

Patient: Samuel Patel

DOB: 01/19/1963 (Age 63)

MRN: MRN-2025-0298

Ordering Physician: Dr. Sarah Williams, MD

Collection Date: 08/10/2026

DIABETES MONITORING:

- Hemoglobin A1c: 10.8% (HIGH) [Ref: <5.7] - Goal <7.0
- Fasting Glucose: 342 mg/dL (HIGH) [Ref: 70-100]

COMPREHENSIVE METABOLIC PANEL (CMP):

- Glucose: 342 mg/dL (HIGH) [Ref: 70-100]
- BUN: 24 mg/dL (HIGH) [Ref: 7-20]
- Creatinine: 1.4 mg/dL (HIGH) [Ref: 0.7-1.3] - Baseline 1.2
- eGFR: 51 mL/min/1.73m2 (LOW) [Ref: >60]
- Sodium: 137 mEq/L (Normal) [Ref: 136-145]
- Potassium: 4.6 mEq/L (Normal) [Ref: 3.5-5.0]
- CO2: 25 mEq/L (Normal) [Ref: 23-29]
- Calcium: 9.4 mg/dL (Normal) [Ref: 8.5-10.5]
- Albumin: 4.2 g/dL (Normal) [Ref: 3.5-5.0]
- ALT: 34 U/L (Normal) [Ref: 7-56]
- AST: 29 U/L (Normal) [Ref: 10-40]
- Alkaline Phosphatase: 82 U/L (Normal) [Ref: 44-147]

URINE MICROALBUMIN/CREATININE RATIO: 118 mcg/mg (HIGH) [Ref: <30]

LIPID PANEL:

- Total Cholesterol: 218 mg/dL (HIGH) [Ref: <200]
- LDL Cholesterol: 138 mg/dL (HIGH) [Ref: <100 - diabetic goal]
- HDL Cholesterol: 34 mg/dL (LOW) [Ref: >40]
- Triglycerides: 232 mg/dL (HIGH) [Ref: <150]

CLINICAL SIGNIFICANCE:

Markedly elevated A1c and fasting glucose indicate severely uncontrolled diabetes. Elevated urine microalbumin with declining eGFR suggests early diabetic nephropathy progression. Dyslipidemia below diabetic targets. Recommend prompt medication re-initiation with renal-adjusted dosing consideration and insulin initiation evaluation. Ketones were not detected on urinalysis; no acidosis pattern present.

Reported by: Quest Diagnostics Core Lab

Verified by: Dr. Michael Torres, MD, Pathologist

Date: 08/10/2026